NCCP 00623 · VERSION 3A

Bevacizumab + capecitabine

Bevacizumab infusion on Day 1; capecitabine twice daily on Days 1–14, then no capecitabine on Days 15–21. Follow your individual prescription.

Draft for clinical, pharmacy and patient/PPI review. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Mouth

Capecitabine

Painful ulcers or reduced intake: stop tablets and call.

2 Chest

Capecitabine

New chest pain: stop tablets and seek urgent assessment.

3 Bowel / stoma

Capecitabine

Significant diarrhoea, increased output or dehydration needs prompt advice.

4 Hands / feet

Capecitabine

Pain or peeling affecting daily activity: stop tablets and call.

5 Blood / infection

Capecitabine

Fever, shivers or sudden illness: contact oncology immediately.

6 Kidneys / urine protein

Bevacizumab

Urine protein is checked; swelling or reduced urine needs advice.

7 Head / blood pressure

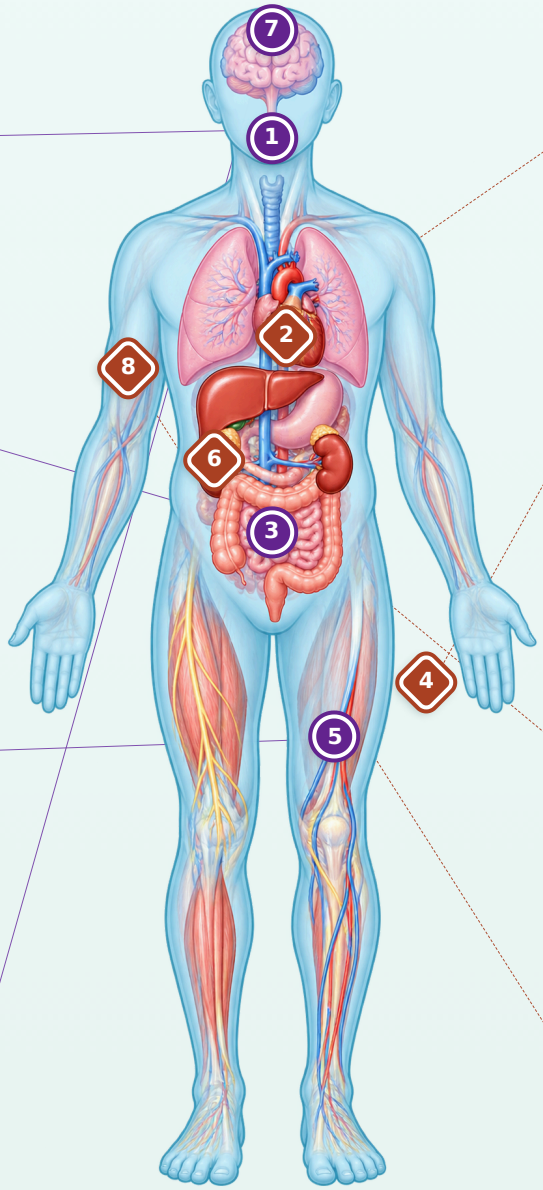
Bevacizumab

Severe headache, visual change or sudden weakness needs urgent help.

8 Bleeding / wounds

Bevacizumab

Report significant bleeding, clots and poor wound healing.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

NCCP 00623 · PRACTICAL SUPPORT

Your treatment, at home.

Important medicine-specific effects

Capecitabine

- Diarrhoea and dehydration
- Painful hands / feet
- Mouth soreness and reduced intake
- Low blood counts / serious infection
- Chest pain / heart problems
- Severe early toxicity, including DPD-related toxicity

Bevacizumab

- High blood pressure / urine protein
- Bleeding or blood clots
- Delayed wound healing
- Rare bowel perforation / fistula
- Rare serious neurological effects
- Infusion reaction

Make the plan practical

Written tablet instructions

Ask for the number of each tablet strength per dose, the start and stop days, and the contact number. This page does not calculate a personal dose.

Missed or vomited dose

Skip that dose; take the next scheduled dose at its usual time unless the team has told you to stop. Never double up. Record missed doses and report repeated vomiting.

Skin, mouth and bowel care

Use the agreed skin and mouth-care plan. Keep your prescribed anti-sickness and diarrhoea medicines available. Significant symptoms still require stopping tablets and calling.

Checks and medicine review

Confirm DPD testing and regular blood tests. Tell the team about warfarin, phenytoin, allopurinol, brivudine and any new medicines or supplements. These need an interaction check.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

CUH · MON–FRI 8AM–5PM

087 181 9645

Acute Oncology Service

CUH · WEEKENDS / BANK HOLIDAYS 9AM–5PM

021 4922000

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Complete reference, visual timeline and passport

https://sactcheck.com/patient/00623/

Sources and review status are available online. Follow your own prescription and hospital instructions.